



ShabbaTTogether

Tips for Those Experiencing a Medical Crisis & Those Watching *Others* Experience a Medical Crisis

Goals:

This is a handout for you to review and use as to help understand how to support someone going through a medical crisis, or for practical ideas for yourself while experiencing a medical crisis.

This handout has two sections:

1. Tips for Families experiencing a medical crisis: **Mindsets to carry with you**

- The “Mindsets” section is focused on ideas that allow a person to recognize that Hashem (G-d) is in charge. **The practical takeaways** are: A) “Moach Shalit al Halev,” (the mind controls the heart) and B) One must have a G-dly “recalibration tool,” a basic idea of our relationship with G-d readily accessible in order to maintain control of the mind when in crisis situations.

2. Tips for *Friends of Families* experiencing medical crisis: **Actions to help**

- The “Action” section is focused on advice that will allow a friend to truly help someone in crisis. **The practical takeaways** are: A) The critical importance of Gemilut Chassadim (Acts of Kindness), and B) One must look at things *solely* from the standpoint of the person in crisis in order to maintain their dignity and serve them appropriately.

Target Audience:

Adults

Time:

20 minutes to read

Medium:

To read at home

Note to Reader:

The following are adaptations from a forthcoming book, titled "Of Medicine, Miracles and Mindsets", written by Rabbi Elie and Chaya Rochel Estrin. The book tells of the remarkable medical journey of a baby named Nissi, a boy originally given a 4-6 hour life expectancy due to severe cardiac defects and other complications. His ultimately triumphant roller coaster ride included 9 surgeries in his first 3.5 years of life, a cardiac arrest at 29 days old, and moments of abject terror, but also of indescribable joy. Drama of the story aside, in the second section of the book, Nissi's parents share their perspectives and lessons learned, for the purpose of helping us help each other, bringing the unity back into the community.

With COVID-19 on everyone's mind, we have all learned that none of us are invincible. Many of us have dealt with significant stress over the past year, with the changes in routine, financial stresses and fears for immunocompromised loved ones foremost on our minds. This is a world that special-needs families know very well.

In their book, the Estrins talk about seven "mindsets" they used to power themselves through their special-needs and medical experiences. These mindsets are valuable for all, as they set a tone for dealing with crisis in a uniquely Jewish manner; a manner which is simultaneously healthy in mind as well as spirit.

Thoughts for Families in Medical Crises:

You've been thrown into a situation that you were never prepared for, and certainly never wanted. Life will, in all probability, never be the same. What do you do?

While everyone may have different coping mechanisms — all in accordance with their lifestyle, belief system, and character traits — here are some thoughts taken from our experiences, written with the heartfelt hope that others will find them useful in finding healthy crisis management that fits their own unique situation.

First, a few mindsets that kept us going. These ideas had been part of our life long in advance of this experience. We somehow made them part of our daily routine to the point that they became automatic guidance points for us, spiritual orientation tools. Simple reflection on these concepts, even for a few seconds a day, proved beyond invaluable to us.

Mindsets:

- As Jews, we say Shema twice daily. On a simple level, we are stating that there is only one G-d. But on a deeper level, we are affirming that in truth, everything is G-d. There is nothing outside of G-d. Cognizance of this truth lifted us up. It reminded us that we were created by G-d and are kept alive by G-d — and so is this child.
- The special relationship between G-d and every individual was described by Rabbi Israel Baal Shem Tov as even greater than the love between an only child born to parents in their old age. Chaya and I took two elements out of this idea. First, Chaya felt the strength of leaning on G-d. Her heart said, “G-d, you love me as a parent loves a child, and therefore, I know I can trust that what I’m going through is what’s best for me, even if I don’t feel it.” Second, during our ordeal, our prayers became plaintive and straight from the heart. They weren’t extravagant; just simple and absolutely raw. I remember thinking, “G-d, this is all I’ve got. Please take it.” In retrospect, I look on those prayers as being possibly equivalent to a *minchas ani*, a pauper’s gift to G-d, which, despite its lack of adornments, is beloved by G-d due to its sincerity and wholesomeness. I do not pretend any holiness or depth of spiritual feeling at the time. I had nothing of the sort. All that remained was simple: “G-d, you created me, and you put me in this situation. Please help.”
- When we asked for help, we took our lead from Chana, the mother of the prophet Samuel. She did not ask for herself; she asked that her prayers be answered *for G-d’s own sake*. Likewise, we begin the thrice-daily *Amidah* prayer with the statement, “G-d, open my lips, and my mouth shall declare Your praise.”¹ Noting that the proper place for such a declaration should be at the very beginning of prayer, the Rebbe explains that prayer until this point should have brought one to a state of self-nullification.² As such, the requests we make in the *Amidah* are no longer for my personal needs or desires, but for the purpose of declaring G-d’s praise. Likewise, we asked G-d’s assistance with our child, not for ourselves, but for the purpose of expressing G-d’s praises.
- Sometime prior to this experience, I came across an unbelievable idea in a Chassidic discourse from the Rebbe.³ In it, he explains that the Hebrew word “*Nes*” has several meanings: a miracle, a challenge, and a banner. The three unrelated homonyms then express a powerful message: The *purpose* of a challenge is to *lift you up* (like a banner). We *overcome* challenges by *lifting ourselves higher*. And the meaning of *challenges*, as is the purpose of *miracles*, is to show the inner truth of the world — nothing exists besides G-d. This concept reverberated in my mind throughout the experience.

¹ Samuel 1:11.

² *Toras Menachem Sefer Hamaamarim* 5711–12, p. 257.

³ *Ibid.*, p. 332.

- “You don’t know how strong you are until being strong is the only choice you have.” Chaya Rochel saw this message on a Cardiac Awareness bumper sticker, and uncharacteristically, she felt compelled to buy it and display it on her car. Every time people tell us they are impressed by our strength, we quote them our bumper sticker (although a quick internet search credits it to Bob Marley).
 - That pithy line has its philosophical sources in Jewish theology. The Talmud tells us that G-d only gives us what we can handle.⁴ While, honestly, sometimes that statement feels ... well, at the least, unrelatable. (And though they mean well, it can even feel downright infuriating when someone tells you, “Boy, you must really be able to handle a lot; after all ...” We can come to that discovery of our inner strengths ourselves, but what we need from others is empathy, not their assumption that we can handle what we’ve been given.)
 - The following paraphrase of an insight from Rabbi Yosef Yitzchak Schneerson of Lubavitch allowed me, personally, to come to terms with it:
Our Sages say that “one is obligated to say the world was created for me.”⁵ The word for world, *olam*, has four distinct meanings in Hebrew: a physical world, concealment, youthful strength, and time. The message in this statement is now deeper and richer: I am obligated to consider and reflect upon my world and to recognize that any seeming negativity that I see at any particular place and time was created for me to uncover the G-dliness hidden within it. At the very same time, within that “concealment” is also locked the strength that I have been given to uncover the inner truth — that it is but a facade over the G-dliness that enlivens it all.⁶
- Within the struggles of fighting for a child’s life, there is one particularly dangerous emotion that can — understandably — arise, but must be avoided at all cost: anger. The Rebbe told pediatrician Dr. Stuart Ditchek the following: “If you are angry at G-d because of a diagnosis, you are hurting the chance that this child has for a miracle.”[1] Frustration? Certainly. Our prayers are full of expressing frustration; we continuously ask G-d to change things as they are. But anger is quite different; it is a destructive flame. It sends us into the spiral of despair, instead of into the depths of our souls, and to the innermost connection with G-d. That space is the place of our core strength, and, when G-d wills it, is also the place from where miracles spring forth. But it doesn't deny the pain; it acknowledges the struggle. As King David writes about G-d (in Psalms 91:15), “I am with him in trouble and pain.” Anger will create but distance when we need to be as close as possible.

⁴ Tractate *Kesubos* 67a.

⁵ Tractate *Sanhedrin*, 37a.

⁶ *Igrot Kodesh Rayatz*, vol. 2, p. 248.

- Finally, here are three meditations that Chaya has found very valuable: First, let go of the past — you can't do anything about it. Second, let go of the future — you haven't gotten there. And third, focus on here and now — because that's all we can control. So much of our stress is from our pain of the past and fear of the future. By eliminating the cold grip they have over us, we release ourselves to make the most of the present.

Ultimately, it really comes down to this: Despite the difficulty inherent in doing so, we need to control and direct our thoughts and emotions, because otherwise, they will distract us from the battle ahead. Most important is to trust G-d and remain positive, not just because that gives us the strength to proceed, but because it creates Divine channels for blessing. When we allow ourselves to fall into despair, we lose sight of our goals and allow ourselves to fall victim to the disease. Practically speaking, it intervenes in our ability to project strength, direction, and fighting spirit to the sick child and to the medical team. You want them to be fighting with you. Your trust, optimism, and projection of calm will definitely influence their attitudes.

Inclusion is everybody's business, because eventually, it becomes everyone's experience. Virtually all human beings will go through an experience of special needs; although we hope and pray that those needs will be minor and controllable. But considering that sobering thought, perhaps it would be valuable to look at things through the eyes of a special-needs family, in order to better assist and adapt ourselves to the reality of those struggling with medical or mental crises. Our people, our family, even we ourselves, deserve nothing less.

In their book, the Estrins compiled this list of tips for those trying to support a family with a child in the hospital, based on what was most effective for them. While we do not wish to suggest that all special-needs experiences are the same, there are certainly certain universals that the authors suggest can be sensitively applied to most people and most circumstances.

Thoughts for Friends of Families Going Through Medical Crises

As we’ve noted, we benefited greatly from the support of our friends and family. Over time, we also learned what elements of assistance were particularly effective for us and what wasn’t helpful. With regard to the latter, our experience was that many people simply did not know what to do to help or how to express their concern. Based on our observations, here are a few tips on how friends and community can best help families in crisis:

Actions you can take to help:

- **Communicate your concern.** If you’re thinking about the person, drop them a line and let them know that. Text or Whatsapp messages are invaluable. Even if the parents do not respond, know that your concern is felt and appreciated.
- **Offer specific help.** While general questions such as “What can I do to help?” are well meaning, for a family in crisis this ends up being yet another overwhelming question that needs to be addressed. Instead, offer tangible and concrete assistance: “I’m going to the grocery store; please text me a list of things you need.” The practical offer gives the person in crisis an idea of how indeed you can help them, while the vague offer often gets the unrealistic, and probably untrue, response, “Everything’s fine.” In addition, a few pointed questions will help you pinpoint the best way you can help this particular family. Asking what they like for supper, when is a good time to send over cleaning help, or similar questions eliminate any need for decision-making to an already overwhelmed family.
- **Express concern for the caregiver, not just the sick family member.** When caring for a sick family member, the caregivers often put their own needs last. It’s okay to ignore the patient for a little while and focus on the exhausted, harried, stressed out, and emotionally spent parent or caregiver. By giving them strength with your friendship and care, you will benefit the patient as well.

To quote a good friend of ours, Dr. Sarah Stroup, “Understand that the parents and caregivers are also patients. We are also suffering. Treat us kindly. Deal with our silence. Deal with our anger. Deal with our outbursts. Deal with what may seem like antisocial behavior — because guess what: we’re not feeling too social, and we may have difficulty around those who have healthy children. We may resent them. That is our right. We are ill too. We are doing our best ... cut us some slack.”

Some helpful ideas and always appreciated gestures are: gift cards of any sort, babysitting and cleaning help, dinners, groceries, toys for the siblings, small gifts for the caregivers, cash, respite care, and meal trains.

- **Be sensitive in conversation.** Sometimes people are overly positive because they can't handle the intensity of the situation, so they try to reassure primarily themselves — and only by extension the family in crisis — by nervously making comments such as, "It's going to be okay", or, "He's doing better now, right?" Make sure that your messages of positivity and hope are sensitive to the feelings of the family.
Families going through medical situations or dealing with a child with disabilities are experiencing very deep emotions, even if they don't show it outwardly. Those who are blessed with good health cannot begin to imagine the life of a special-needs family — the chaos that can erupt at a moment's notice, the never-ending work to keep hospitalizations at bay, and the sheer emotional exhaustion from it all. Your appreciation of the magnitude of these and other challenges — not in a heroic way, but with an eye towards reality — will be greatly welcome. Common expressions like "I know what you mean" can be painful to a caregiver under the pressure of life and death decisions. Train yourself to replace it with, "I can't even imagine" or similar. The difference between sympathy and empathy is of utmost importance. Sympathy is when you feel bad for the other person. Empathy is when you join the other to *share* their burden. Validate instead of relate.
- **Don't probe for information.** The caregivers will convey whatever they feel comfortable sharing.
- **Leave the medical advice to the medical experts, unless asked.** You probably don't really know or understand the details of this individual case, and sometimes, alternative "it can't hurt to try" methods really can harm.
- **Don't offload your own trauma.** For those of us who have gone through similar experiences, this is an important tip. Success stories and shared experiences are often a beacon of light and hope. At the same time, be mindful not to offload your own story and trauma without being prompted to do so. Keep in mind that until the family is out of the woods, they are not always capable of listening to the details of others' experiences. They will certainly not be capable of helping you process or heal from your own traumatic hospitalizations...

At the same time, both during our preemie and cardiac kids' hospitalizations, we hung onto success stories. We wanted to hear of the children who beat the odds. We'd look at the pictures of NICU graduates in the fervent hope that our Little Guy would join them some day. But we were not yet capable of listening to the details, and we certainly were not capable of comforting those who'd gone through traumatic hospitalizations! Mostly, we needed our own somewhat unclear feelings to be validated and sifted through.

The key is to be a sensitive listener and to provide heartfelt encouragement and strength. Don't be a taker; don't absorb energy from someone who needs every ounce of it. Remember, you are there to *lighten* the burden.

Here are some keys to hospital visiting:

- **Coordinate with the family beforehand**, and then double check when you get to the hospital or home. In medically complex situations, and particularly in the hospital itself, things are often dynamic or unpredictable, so be flexible.
- **Make sure the family knows exactly who is coming**, and that they are okay with it. Sometimes, well-meaning friends bring along a toddler or baby, and it causes unnecessary stress; it may be hard to see a healthy child when theirs is fighting for his life. Other times, the family may be worried about germs, or that the rambunctious three-year-old might pull on or trip over a vital cord or tube. It might also be fine — just clear it with them. No surprises.
- **Visits don't need to be long**. Some people stress out so much about their visit that they find it too hard to stop by ("What will I say? What will I do?"). But the truth is that even ten minutes is valuable; in fact, more than that might not be. Unfortunately, sometimes people create more exhaustion by overstaying their welcome. But when you make it into a short, sweet experience, just enough to express "I care and I'm with you," you will have brought far more than you can possibly believe to the equation in just a short amount of time.
- **A hot meal goes a long way**. Hot soup, in particular, is incredible. We rarely ate a proper, fresh, hot meal when we were in the hospital, and if we did, it was only because a friend brought it to us...

We know we couldn't have done it without our friends and family. For them, for all the above, and for life itself, we are eternally grateful.

"Of Medicine, Miracles Mindsets" will be published by Mosaica Press. The book is due to be out in fine Judaica book stores worldwide and Amazon by Spring 2021.

Rabbi Elie and Chaya Estrin co-directed Chabad at University of Washington for 13 years. Following the birth and subsequent hospitalizations of their son Nissi, the Estrins moved to North Miami Beach, FL. Rabbi Estrin currently serves as the Military Personnel Liaison for the Aleph Institute - the Chabad representative responsible for caring for Jews in uniform in the US Armed Forces – and teaches in the Chaya Aydel Seminary of Hallandale Beach. Rabbi Estrin is also a chaplain in the US Air Force Reserve, with the rank of Captain-Promotable. When not serving as Nissi's medical care coordinator, multidisciplinary therapist and chauffeur, Chaya Estrin works in data management. The Estrins are blessed with 6 children.